



Guidelines

Alcohol and Other Drugs: Management of Addiction, Toxicity, and Withdrawal

Scope

Site	Service/Department/Unit	Disciplines
SCGOPHCG	Organisation Wide	Medical, Nursing, Allied Health, Pharmacy,

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Introduction

Early detection of withdrawal and the prevention of risks associated with withdrawal are key components of effective and safe patient management. This document was developed in collaboration with “walk with me guidelines” (Refer to [Pathways to alcohol and other drug early intervention and withdrawal management](#)) to establish the minimum practice standards for the care of patients who may be affected by alcohol or other drugs.

Information is provided specific to alcohol or drug group including initial considerations for patient care, signs and symptoms of intoxication and withdrawal, assessment, and management.

Risk Statement

Non-compliance with this guideline will:

Breach legislative requirements	☐	Impact on Patient Quality of Care	☒
Breach National/State/Hospital Policy	☐	Impact on Patient Safety	☒
Breach professional standards	☐	Misconduct	☐
Breach SCGOPHCG Mission & Values	☐	Staff Safety	☐
Other:	☐		

Definitions

Alcohol	Alcohol is a central nervous system depressant/sedative drug. Even in small doses alcohol significantly impedes motor and cognitive function depending on the dose and individual susceptibility.
Clinical Advisory Service (CAS)	24/7 Phone line for all matters related to Community Program for Opioid Pharmacotherapy (08) 94425042.
Drug	Any substance (except for food and water) which, when taken into the body, alters the body's function either physically and/or psychologically. Drugs may be legal e.g., alcohol, prescription medication or illicit e.g., methamphetamine
Intoxication	A condition that follows the administration of a psychoactive substance and results in disturbances in the level of consciousness, cognition, judgement, affect, or behaviour, or other psychophysiological functions and responses.
Withdrawal state	A group of symptoms of variable clustering and severity occurring on absolute or relative withdrawal of a psychoactive substance after persistent use of that substance (ICD-10). The onset and course of the withdrawal state are time-limited and are related to the type of psychoactive substance and dose being used immediately before cessation or reduction of use. The withdrawal state may be complicated by convulsions.
Wernicke's Encephalopathy (WE)	An acute reversible neuropsychiatric condition which presents in alcohol dependent individuals due to acute thiamine deficiency. Acute WE are characterised by the classical triad of symptoms of cerebellar ataxia, ocular abnormalities, and a state of confusion.
Korsakoff's Syndrome (KS)	A largely irreversible condition characterised by short term memory loss, decreased learning ability and compensatory confabulation, due to deficiency of thiamine. This is a progression of untreated or inadequately treated WE.
Drug and Alcohol Clinical	A specialised telephone consultancy service that provides clinical advice to health professionals across WA on all issues relating to

Advisory Service (DACAS)	management of patients with alcohol and other drug use. This service is available 08:00 to 20:00 hrs Monday to Friday. Phone (08) 6553 0520.
Urine drug screen (UDS)	A rapid, visual panel of immunoassays that can be used for the simultaneous qualitative detection of multiple drug metabolites in human urine. Widely accepted method for alcohol and other drug screening.
Clinical Institute Withdrawal Assessment for Alcohol (CIWA-Ar)	A proven, valid, and sensitive instrument for assessing the clinical course of simple alcohol withdrawal. It assists clinicians to assess the severity of symptoms and timing of assessments to guide treatment appropriately.
Clinical Opioid Withdrawal Scale (COWS)	A clinician-administered instrument that rates eleven common opiate withdrawal signs or symptoms
Delirium tremens (DT)	Is defined by hallucinations, disorientation, tachycardia, hypertension, hyperthermia, agitation, and diaphoresis in the setting of acute reduction or abstinence from alcohol. Approximately 5% of patients who undergo withdrawal from alcohol suffer from DT.
DSM-5 Classification of Substance/Alcohol Use Disorder	• Recurrent substance uses in situations where it is physically hazardous. • Recurrent substance use resulting in a failure to fulfil major role obligations at work, school, or home. • Continued substance use despite having persistent or recurrent social or interpersonal problems caused or exacerbated by the effects of the substance. • Craving or a strong desire or urge to use the substance. • Substance is taken in larger amounts or over a longer period than was intended. • There is a persistent desire or unsuccessful efforts to cut down or control substance use. • A great deal of time is spent in activities necessary to obtain the substance, use the substance, or recover from its effects. • Important social, occupational, or recreational activities are given up or reduced because of substance use. • Substance use is continued despite knowledge of having a persistent or recurrent physical or psychological problem that is likely to have been caused or exacerbated by substance use. • Tolerance, as defined by either: ○ a need for markedly increased amounts of substance to achieve intoxication or desired effect or ○ a markedly diminished effect with continued use of the same amount of the substance. • Withdrawal, as manifested by either: ○ the characteristic withdrawal syndrome for the substance or ○ the substance (or a closely related substance) is taken to relieve or avoid withdrawal symptoms.

General Principles

Alcohol and other drugs (AoD) usage screening should occur in pre-admission clinics or during assessment in the Emergency Department (ED), where possible.

Many individuals who use AoD have co-occurring mental health issues. Patients presenting with either AoD issues or mental health issues should be assessed and treated concurrently¹.

Withdrawal management may only be the first stage of alcohol and other drug treatment and by itself does little to change long-term drug dependence. Patients will be encouraged to continue drug treatment following withdrawal¹.

All patients at risk of experiencing alcohol withdrawal are to be commenced on an Alcohol Withdrawal Chart (CIWA-Ar).

It is mandatory that all patients experiencing acute alcohol withdrawal are provided with thiamine replacement therapy. Refer to the [SCGOPHCG Thiamine Treatment for Alcohol Dependent Patients Medication Management Guideline](#).

- All patients are to be advised on admission that the use of alcohol and/or illicit drugs will not be permitted whilst they are in hospital.
- If there is reasonable concern that a patient has consumed AoD as an inpatient they may be requested to submit to breath, urine and/or blood alcohol screening.
- All breathalyser testing must be performed by appropriately trained and experienced clinicians, with relevant medical staff consulted in the decision to test the patient.
- The [Medicines and Poisons Act 2014](#) requires authorised medical practitioners / nurse practitioners to report to the Department of Health (DoH) Medicines and Poisons Regulation Branch within 48 hours, when they have reason to believe a person has acquired, because of repeated administration of drugs of addiction, an overwhelming desire for the continued administration of a drug of addiction. Reports of dependence will be made using the approved 'Reporting a Drug Dependent Person' or 'Reporting an Oversupplied Person' form.
- Prior written authorisation must be received from the DoH before prescribing Schedule 8 discharge medicines to a person who has been recorded as a Drug Dependent Person.
- [Health Practitioner Regulation National Law \(WA\) Act 2010](#) requires a psychiatrist to notify the DoH Medicines and Poisons Regulation Branch within 72 hours of making a diagnosis of stimulant induced psychosis. This is made by completing the 'Notification Stimulant Induced Psychosis' form.

1. Screening, Assessment, and Withdrawal Management

1.1 General withdrawal management guidelines

Health Service Providers (HSP) clinicians are most likely to encounter patients requiring AoD withdrawal management in one of the following situations:

- **Acute Presentations:** Patients presenting with established AoD withdrawal symptoms, or patients presenting with issues related to acute intoxication or overdose.
- **Unplanned Withdrawal:** Patients with AoD dependence who present for a related or unrelated health condition and are at risk of withdrawal during admission.

1.2 Assessment for withdrawal management

Components of assessment for withdrawal management include:

- medical and psychiatric history and examination including a full medication history

- substance consumption history, including time and date of last use and history of poly- substance use
- history of withdrawal complications
- underlying psychosocial factors that may contribute to dependence or complicate withdrawal management.

HSP clinicians provide short-term withdrawal management on an inpatient basis for patients presenting with acute withdrawal symptoms and those who develop unplanned withdrawal whilst in our care for another health reason.

If documenting a full substance consumption history is not initially feasible:

- obtain whatever history is available, especially details of use within the last three months
- identify signs of substance consumption and effects during examination
- take and document a full consumption history as soon as possible

Predictors of severe or complicated withdrawal may include:

- heavy, recent consumption patterns - dose, regularity, and duration of use
- heavy or regular use of multiple substances
- co-existing medical or psychiatric illness
- past severe withdrawal experience.

Patients with any of these factors may be at higher risk of severe or complicated withdrawal and may benefit from early referral to AoD Clinicians to assist with assessment and withdrawal management.

- Many consumers with AoD issues have concurrent mental health issues.
- Patients presenting with either psychiatric illness or alcohol and other drug-related presentations should have a comprehensive assessment for both and be treated accordingly.

1.3 Screening and Assessment Initial Considerations (SBIRT)

- Screening, Brief Intervention, and Referral (SBIRT) is an evidenced-based comprehensive approach used to identify, intervene, and provide appropriate referrals for individuals who are at risk of or experiencing substance use disorders practiced by AoD clinicians at SCGOPHCG. Its implementation is aligned with the broader goals of the [National Drug Strategy](#), which focuses on prevention, harm reduction, and improving access to treatment and support for individuals affected by drug-related issues.
- The patient's mental health and current form of thought, perception and behaviour may be affected by recent substance use e.g., intoxication/withdrawal, psychosis, impaired cognitive function, suicidal/homicidal ideation.
- Longer periods of use may complicate treatment and compromise recovery goals.
- The patient's AoD use may impact their legal competency in relation to consent to treatment.
- Issues with AoD can affect anyone. Some groups in the community are more at risk than others:
 - People with mental health conditions
 - People with a history of trauma who may be seeking 'emotional blockade'
 - Pregnant people
 - People with poly-substance use
 - People living in remote and very remote areas²
 - Aboriginal & Torres Strait Islander Peoples who may have:

- been impacted by government policies³
 - trauma in its many forms from past policies causing post-traumatic stress
 - intergenerational trauma which may result in substance misuse
 - limited access to culturally appropriate care and treatment
- Lesbian, gay, bisexual, transgender, intersex (LGBTIQA+) communities⁴
- Young people or older adults
- People experiencing domestic violence
- People with acquired brain injury or cognitive impairment
- Homeless people or people with housing instability
- Culturally and linguistically diverse people - consider referral to:
 - appropriate Liaison Services
 - interpreter services can be access through SCGOPHCG [Language Services](#)

1.4 AoD Screening

- A simple routine AoD screen occurs for all patients seen in any setting excluding patients with altered conscious state or acute clinical condition
- Initial screening identifies those patients at risk of withdrawal and whether monitoring using a CIWA-Ar or COWS is clinically indicated.
- A standard drinks guide is provided to assist in AoD screening. [Refer to Appendix 1.](#)

The CIWA-AR should be commenced where any predictors of alcohol withdrawal are identified.

The COWS should be commenced where any predictors of opioid withdrawal are identified.

1.5 AOD Assessment

- AoD clinicians use the evidenced based biopsychosocial model for assessing patients. Issues that are related to and can exacerbate and prolong withdrawal include:
 - Mental health
 - Legal
 - parenting/family/child protection
 - housing
 - domestic violence
- Patient history taking will include:
 - blood borne viruses (BBV)
 - sexually transmitted infections (STI)
 - pregnancy
 - seizure history
 - physical co-morbidities
 - mental health co-morbidities
 - social history including dependents, housing
 - cognitive assessment.
- physical examination
 - evidence of intoxication
 - evidence of withdrawal
 - evidence of complications from substance use.

- Laboratory and other testing will include:
 - full blood count
 - serum electrolytes including potassium, magnesium, and phosphate
 - glucose
 - creatinine
 - liver function tests
 - lipase
 - blood alcohol level
 - urine drug testing
 - beta human chorionic gonadotropin (HCG).

1.6 Follow up/ Discharge planning with AoD service

- Involve patient/family in discharge planning informing them of their options in the community where these supports are available.
- Facilitate engagement with community and/or specialist support service providers who will provide ongoing care where possible
- Include information in the discharge plan on how the patient / family can seek additional support including return to the current service.
- Provide information on strategies to manage and reduce risks or harms associated with continued substance use.
- The patients discharge summary must be completed within 24 hours of discharge and forwarded to the patients GP.

1.7 Referral to specialist services

- Referrals to specialist [SCGOPHCG AoD Service](#) will be initiated when clinically indicated.
- For SCGOPHCG patients:
 - ED patient can be referred through the Emergency Department Information System (EDIS), by phone, direct consult, or email SCGH.ADS@health.wa.gov.au.
 - Inpatient wards and Outpatient referrals are via phone call, direct consultation, e-Referral, and email SCGH.ADS@health.wa.gov.au
 - **AoD Service can be contact on 6475 6323. Working hours are 0630-1500 (Monday – Friday) 0800-1600 (Saturday). A/H patient referrals will be followed up the next working day.**

For more details refer to [SCGOPHCG Alcohol and Drug Service \(ADS\) Hub page](#)

1.8 Referral to C-L Psychiatric Service CNC

- Referrals to C-L psychiatry Service CNC (Adult: 0435572264; Older Adult 65+ 0404828837) between 0830-1630 Monday to Friday.
- Complete eReferral

2. Substance Specific Quick Reference Guide

Substance-specific withdrawal management Quick Reference Guides have been developed for:

- alcohol withdrawal management ([Appendix 2](#))
- benzodiazepine withdrawal management ([Appendix 3](#))
- cannabis withdrawal management ([Appendix 4](#))
- opioid withdrawal management ([Appendix 5](#))
- amphetamine and amphetamine-type substances withdrawal management ([Appendix 6](#))

These Quick Reference Guides have been designed to provide a brief set of substance-specific guidelines for safe management of AOD withdrawal and should be tailored to individual requirements. These reference guides also present the level of evidence associated with the recommendation. Refer to [Appendix 7](#) which outlines the classification schemes for categorisation/grading of evidence.

If the patient's symptoms are not improving despite implementing these recommendations, seek assistance from your AOD Clinicians (where available) or the Drug and Alcohol Clinical Advisory Service.

The **Drug and Alcohol Clinical Advisory Service (DACAS)** is a specialist telephone consultancy service that provides clinical advice to health professionals across WA on all issues relating to management of patients with alcohol and other drug use (excluding **Community Program for Opioid Pharmacotherapy CPOP**).

The service is provided by experienced Next Step Addiction Medicine Specialists and is available to health professionals across Western Australia.

DACAS operates from 8:00am to 8:00pm Monday to Friday. After hours, a message can be left, and the call will be returned on the next business day.
Phone (08) 6553 0520.

The diagnosis of a withdrawal state is **clinical**, based on history, examination, and clinical progress over time (*see section 1.2- Diagnosis of alcohol and other drug withdrawal*). A withdrawal syndrome may occur in some consumers with alcohol or other drug-dependence who stop or reduce their alcohol or drug use. Note that many patients who undergo unplanned withdrawal within the hospital environment may experience complex withdrawal issues complicated by co-existing physical or psychiatric illness.

These Quick Reference Guides include reference to standardised withdrawal monitoring scales. Such withdrawal scales are **not diagnostic** of withdrawal as many other medical and psychiatric conditions may cause similar symptoms and physiological signs.

Charts based on standardised withdrawal monitoring scales allow assessment of severity of withdrawal symptoms, track clinical progress over time and may help to guide pharmacotherapy and symptom management. Charts should be interpreted in the context of the patient's clinical situation, including co-existing acute illness and co-morbid physical and psychiatric diagnoses. They do not replace other hospital protocols and charts warning of acute clinical deterioration and standard responses to clinical deterioration (e.g., MET calls) should still be followed.

3. Alcohol

3.1 Intoxication signs and symptoms

- Intoxication can be exacerbated by the intake of other medications. Refer to [Appendix 8](#) Drugs with the potential to interact with alcohol.
- Refer to Quick Reference ([Appendix 2](#)) Guide for signs and symptoms of alcohol intoxication

3.2 Clinical management

- **Refer to AoD service 6457 6323 for withdrawal risk and assessment**

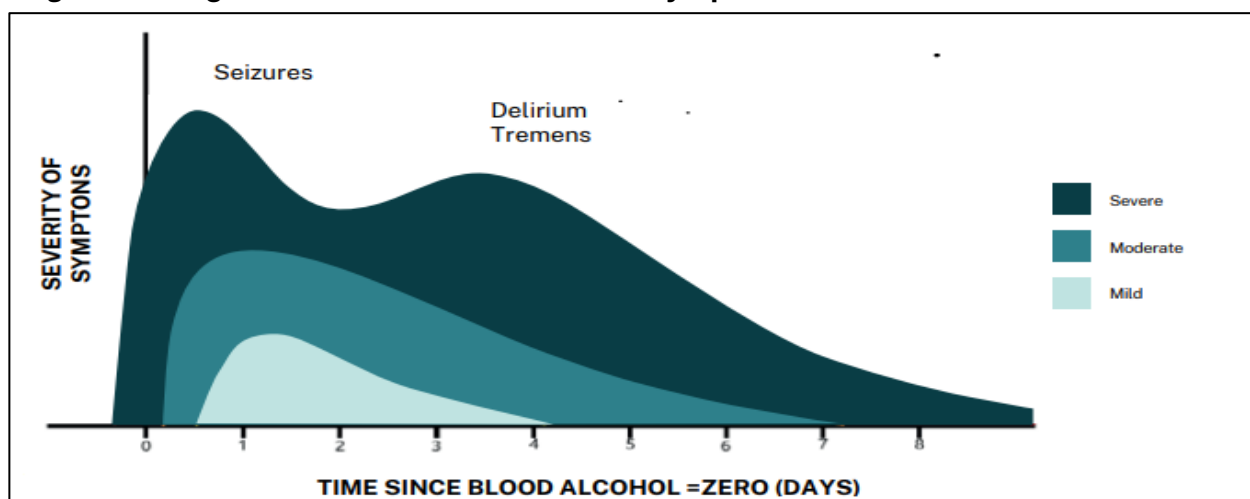
- Clinical management will be based on the clinical condition of the patient and the escalation protocols on the appropriate Observation and Response Chart and CIWA-Ar and in consultation with the Medical Officer (MO)/Senior Clinician and the AoD service
- The level of intoxication will be assessed by breath alcohol reading or blood alcohol concentration test.
 - Alcohol blood levels reduce by 0.015 – 0.02 % per hour on average.
- Assess mental state and discuss any concerns with the MO/Senior Clinician.
- Where possible reduce environmental stimuli e.g., provide single room, dim lights.
- Medical review must occur if there is any identified change to the patient's neurological observations and assessments including Glasgow Coma Scale, speech, eye movements and pupil abnormalities.
- Monitor urine output if there is evidence of potential renal impairment.

3.3 Alcohol withdrawal

- Initial considerations:
 - The signs and symptoms of alcohol withdrawal may be confused with other factors that may be present in critical care patients such as electrolyte imbalances, pain, and infection. *Refer to Table 1.*
 - The risk of major withdrawal is greater in people with acute medical conditions (e.g., trauma or sepsis).
 - Onset of alcohol withdrawal is usually 6-24 hours after the last drink and can occur before alcohol level is <0.01% or breath alcohol is 0.000%.
 - In people with severe alcohol dependence withdrawal can occur when the blood alcohol level is decreasing, even if the patient is still intoxicated. *Refer to Table 2.*
 - Seizures affect 5% of patients, occurring early usually 7-24 hours after the last drink.
- Seizures are tonic-clonic in type i.e., generalised, not focal and usually (though not always) occur as a single seizure.

Table 1: Signs and symptoms of alcohol withdrawal classification

Withdrawal	Autonomic hyperactivity	Gastrointestinal features	Cognitive and perceptual changes
Uncomplicated	Sweating Tachycardia Hypertension Tremor Fever (generally < 38°C)	Anorexia Nausea Vomiting Dyspepsia Diarrhoea	Poor concentration Anxiety Psychomotor agitation Disturbed sleep Vivid dreams
Severe & complex	Dehydration and electrolyte disturbance		Seizures Hallucinations or perceptual disturbances (visual, tactile, auditory) Delirium

Figure 1. Stages of alcohol withdrawal and symptoms⁵**Table 2. Predictors of alcohol withdrawal severity**

Predictor	Severity
Current drinking patterns	- The severity of withdrawal is only moderately predicted by the amount of alcohol consumed. - Chronic heavy alcohol use (15 standard drinks or more per day) is associated with greater withdrawal severity than lower levels of use. - A predictor of increased alcohol withdrawal severity is the onset of withdrawal symptoms (e.g., tremor, nausea, anxiety) upon waking that are normally relieved by early morning drinking. - Individuals with heavy but irregular alcohol use (2-3 days per week), sometimes referred to as binge drinking, generally do not experience severe withdrawal. - Increased risk of cognitive impairment, respiratory depression, and overdose.
Past withdrawal experience	Patients with a history of severe alcohol withdrawal syndrome (e.g., severe anxiety, seizure, delirium, and hallucinations) are more likely to experience similar complications in future withdrawal episodes.
Polysubstance use	- Patients with a heavy or regular use of other substances (e.g., benzodiazepines, stimulants, opioids) may experience more severe and complex withdrawal features. - Withdrawal from both alcohol and benzodiazepines may increase the risk of withdrawal complications. Refer Appendix 8 Drugs with the potential to interact with alcohol.
Medical or psychiatric co morbidity	Patients with concomitant medical conditions (e.g., sepsis, epilepsy, severe hepatic disease, head injury, pain, and nutritional depletion) or psychiatric conditions (e.g., anxiety, psychosis, or depression) could be more likely to experience severe and complex withdrawal complications.

3.4 Acute alcohol delirium

- Acute alcohol withdrawal delirium can be difficult to distinguish from other forms of delirium and in the absence of a comprehensive history, alcohol withdrawal may go untreated. Refer to the [SCGOPHCG Cognitive Impairment](#) hub page.

Delirium tremens (DT) is a medical emergency and typically begins between 48 and 96 hours after the last drink and may last 1-5 days (or longer).

- Acute alcohol withdrawal delirium can be difficult to distinguish from other forms of delirium and in the absence of a comprehensive history, alcohol withdrawal may go untreated. Refer to the SCGOPHCG Cognitive Impairment hub page.
- DT is predicted during alcohol withdrawal by CIWA-Ar scores above 15 - especially in association with a systolic blood pressure >150mmHg, or a pulse rate > 100 beats per minute
- Patients who develop DT will generally require intravenous (IV) fluids and IV sedation and should be managed in a high dependency unit or intensive care unit^{6,7}.
- Alcoholic hallucinations develop within 12 to 24 hours of abstinence and typically resolve within 24 – 48 hours (which is the earliest point at which DT typically develops)^{6,7}.
- Effective alcohol withdrawal management is through symptom identification and treatment. It aims to prevent:
 - Wernicke's encephalopathy
 - progression to severe withdrawal
 - patient falls
 - injury to self and others due to mental state
 - dehydration
 - electrolyte and nutritional disorders
 - potential for seizures
 - complications due to underlying illness

3.5 Pharmaceutical management

- Benzodiazepines are the treatment of choice for alcohol withdrawal due to good oral absorption, rapid onset, and prolonged duration of effects⁸.
- Benzodiazepines should not be abruptly ceased but slowly reduced in a strict regime. Consider specialist advice re titrating regime and risk reduction strategies such as staged supply.

Use caution prescribing benzodiazepines to patients with respiratory failure, elderly patients, liver failure patients or those with a recent head injury. Consult with AoD Service and with a senior doctor to ensure correct dosing.

- The three most used benzodiazepine regimens are symptom triggered therapy, loading dose therapy and fixed schedule therapy.
- The setting in which the withdrawal is occurring will be considered when deciding on the treatment regime.

3.6 Thiamine replacement therapy

Refer to the SCGOPHCG [Thiamine Treatment for Alcohol Dependent Patients Guideline](#)

3.7 Quick Reference Guide

[Refer to Appendix 2](#) for withdrawal pharmaceutical management.

4. Benzodiazepines

Benzodiazepines	Benzodiazepines are effective anxiolytic hypnotic medications. They are also effective amnestic agents and may have a disinhibitory effect especially at high doses.
Clinical Advisory Service	Is a specialist telephone support service provided by Next Step for health professionals providing clinical advice regarding the management of patients with AOD concerns.
Poly-substance use	The use of two or three types of substances at the same time by a person.
Prescription shopping	When a patient visits multiple doctors for the purposes of procuring prescription medication without disclosing other consultations and/or prescriptions.
Prescription shopping programme (PSP)	A 24-hour prescription shopping information and alert service which assists prescribers to make informed prescribing decisions and better manage health outcomes for patients. A patient will meet the PSP criteria if within any three-month period they have been supplied with: - any PBS items prescribed by six or more different prescribers and /or - a total of 25 or more PBS target items and/or - a total of 50 or more PBS items (target and non-target).

4.1 Initial consideration

- Benzodiazepines are used in clinical practice for sedation and the management of anxiety, seizures, withdrawal states, insomnia and drug associated agitation.
- Benzodiazepines should only be used for short term treatment. Dependence develops relatively quickly, though this varies with each patient. Within 2-3 weeks of use 10-20% of patients may become dependent and this increases to 20-45% after more than a year of use⁹.
- Tolerance of different effects of benzodiazepines develops at different rates i.e., tolerance of sedative effects begins after two or three days and is significant by two to three weeks.
- Benzodiazepine use can result in a physical and psychological dependence and can be used in combination with other drugs.

4.2 Therapeutic dependence

- Many people take benzodiazepines prescribed from a single doctor for many years without dose escalation for management of anxiety or insomnia.
- Patients who use benzodiazepines may have difficulty ceasing use because they wish to avoid withdrawal and/or a return of the initial physical and/or psychological issues of concern.
- Risks associated with dependence are poorly defined however it is known that long-term use is associated with an increased rate of cognitive decline and risk of falls. In this patient cohort the risk of continued use needs to be balanced against the distress and benefits of withdrawing.
- Patients in hospital for other reasons may undergo benzodiazepine withdrawal from even low doses of their regular long-term benzodiazepines. Dependant on use people may develop delirium and are also at risk of seizures during withdrawal.
- Withdrawal is best managed by very slow withdrawal supervised by the patient's General Practitioner (GP). Advice on strategies for optimising sleep and controlling anxiety should also be given.

4.3 Screening and assessment

- **Refer to AoD service 6457 6323 for withdrawal risk and assessment**
- At assessment it is critical to identify the patient's use of benzodiazepines to determine how to manage their treatment / withdrawal.
- A history of benzodiazepine use is critical including collateral information from the GP, the PSP and MyHR. The AoD clinician can liaise with external providers to gather this information.
- The patient may be intoxicated on presentation which may impact their ability to provide and receive information.
- Assessment of intoxicated individuals should be limited to ensuring their safety. Caution should be taken when prescribing medications.
- When a patient meets the PSP criteria, verbal advice or a patient summary report on the PBS medicines dispensed and the number of prescribers involved during a defined period can be supplied.
- Further information on the PSP is available on the Australian Government Department of Human Services website.

4.4 Intoxication signs and symptoms and withdrawal management - [Appendix 2](#)

- The patient will be provided with clear information advising the aims of withdrawal and informing them they may feel uncomfortable and experience withdrawal symptoms.
- The reduction plan should be continued on discharge and the benzodiazepine dispensed daily with supervision at their local pharmacy.

5. Cannabis

Also refer to [Appendix 4](#)

Cannabis	The generic name given to the collection of materials derived from the plant cannabis sativa. Delta9-tetrahydrocannabinol (THC) is the psychoactive cannabinoid. It is classed as a hallucinogen as well as a depressant.
Cannabis hyperemesis syndrome (CHS)	A clinical syndrome of cyclical nausea and vomiting occurring after regular cannabis use. The mechanism of CHS is not well understood however, it is associated with regular cannabinoid use rather than being a withdrawal syndrome and is typically relieved with hot showers.
Psychotogenic	A substance that may cause a psychotic reaction.

5.1 Initial considerations

- Cannabis is considered psychotogenic, and intoxication and withdrawal can be associated with precipitating or exacerbating underlying psychosis¹³.
- Underlying psychiatric illnesses or symptoms, which may have been masked during cannabis use, could emerge during the withdrawal period.
- Cannabis can be taken orally or can be smoked, often with tobacco, and the pharmacokinetics and pharmacodynamics of THC vary by route of exposure.
- Inhaled marijuana:
 - onset of psychoactive effects occurs rapidly with peak effects felt at 15-30 minutes and lasting up to four hours.
 - effects will mirror the plasma THC concentrations.
 - Approximately 2-3 mg of inhaled THC is enough to produce drug effects in a naive user

- Ingested marijuana:
 - onset of psychoactive effects occurs rapidly with peak effects felt at 15-30 minutes and lasting up to four hours.
 - effects will mirror the plasma THC concentrations.
 - has a comparatively delayed onset of psychoactive effects that ranges from 30mins to three hours and clinical effects may last up to 12 hours.
 - is less potent due to chemical degradation in gastric acid and poorer absorption.
 - approximately 5-20mg of ingested THC is sufficient to produce drug effects in a naïve user.
- Several factors will influence how soon and for how long someone will feel intoxicated including patient weight, metabolism, gender, eating habits, tolerance levels, medications and other drugs
- Cannabis contains as many carcinogens as tobacco, deposits a third more tar and is thought to be a risk factor for oropharyngeal and lung cancer and coronary heart disease.

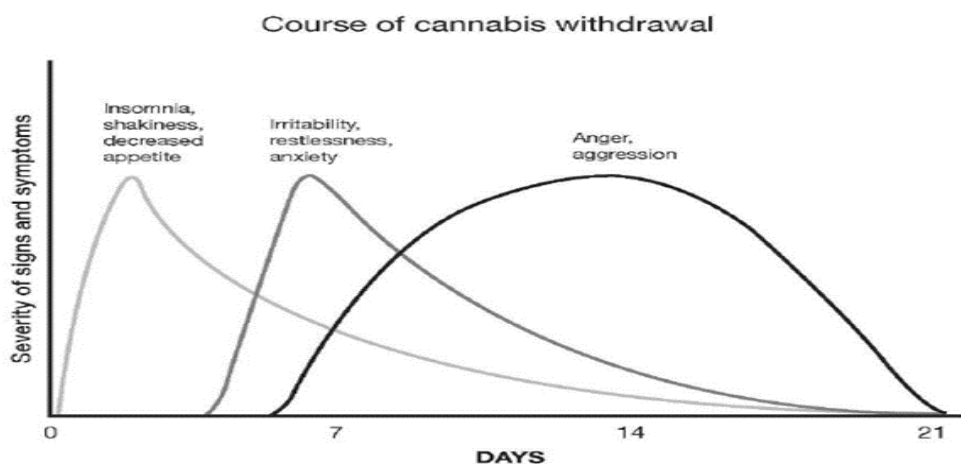
5.2 Intoxication/withdrawal Signs and Symptoms and withdrawal management

Refer to [Appendix 4](#)

5.3 Assessment and management of intoxication

- **Refer to AoD service 6457 6323 for withdrawal risk and assessment**
- The patient may be intoxicated on presentation, and this may affect their ability to provide and receive information. Where possible identify as accurately as possible:
 - dose, frequency, intensity, duration of use
 - method of consumption (smoking/ingesting)
 - poly drug use
 - co morbid physical or mental health issues
 - combination with nicotine – will also withdraw from nicotine
- Consider differential diagnosis or co-ingestion of other substances if severe symptoms persist.
- The management of cannabis intoxication involves supportive care and depends on clinical presentation.
- Perform and document vital signs on the appropriate Observation and Response Chart.

Figure 4. Course of cannabis withdrawal



- Most withdrawal symptoms:
 - emerge one to two days after last use
 - peak within the first seven days
 - continue for approximately seven to 14 days¹⁴.
- Different symptoms may peak at different times throughout the withdrawal e.g., anxiety, anger, and aggression.
- Cannabis withdrawal scales are not used at SCGOPHCG as there is little research to support their validity, reliability, and predictive value.
- Cannabis withdrawal syndrome may last for several weeks, and patients should be referred to a drug and alcohol service for ongoing support.

5.4 Cannabis withdrawal management in the inpatient setting

- For cannabis withdrawal in the inpatient setting refer the patient to the appropriate AoD service.
- Many cannabis users will be able to stop cannabis use abruptly without experiencing any significant physical withdrawal symptoms.
- Provide symptomatic relief as required, most clinical practice involves the use of benzodiazepines

Table 3. Pharmaceutical treatments for symptomatic relief ⁶

Cannabis Withdrawal Symptoms	Suggested pharmaceutical treatments for symptomatic relief
Nausea & vomiting	• Ondansetron PO/sublingual/IV 4-8mg TDS PRN. • Metoclopramide PO/IM/IV 5-10mg TDS/PRN • Prochlorperazine PO 5mg, every 4-6 hours as required
Muscle aches & pains and headaches	• Paracetamol 1g QID PRN PO
Agitation and/or anxiety, restlessness, irritability	• Diazepam PO 5mg up to QID PRN (adjust dose to clinical need and risks) • Acute short-term use only, up to five days
Insomnia	• Temazepam 10mg nocte PRN PO • Maximum of seven days treatment
Abdominal cramps	• Hyoscine butylbromide 10-20mg TDS PRN PO/IV
Symptoms of psychosis e.g. thought disorders, paranoia, perceptual disturbances	• Refer to C-L Psychiatry Service CNC for consideration of Olanzapine (Adult: 0435572264; Older Adult 65+ 0404828837) between 0830-1630 Monday to Friday.

5.5 Cannabis hyperemesis syndrome (CHS)

- Signs and symptoms of CHS and management. Refer to Quick reference guide: Cannabis. Refer to [Appendix 4](#)

6. Opioids

6.1 Use of opioids

- Prolonged use of opioids can lead to the development of tolerance and dependence¹⁶.
- Cessation after prolonged use is associated with a range of withdrawal symptoms. Withdrawal syndrome is uncomfortable but not usually life threatening¹⁶.

- Tolerance to opioids decreases rapidly following withdrawal which increases the risk of opioid overdose if the client resumes prior levels of opioid use¹⁶.
- Harm minimisation advice and ongoing support are crucial to reducing the likelihood of early relapse with potentially fatal consequences.
- Patients not ready for complete withdrawal should consider an opioid substitution treatment program.

6.2 Intoxication signs and symptoms and withdrawal management ([Appendix 5](#))

- **Refer to AoD service 6457 6323 for withdrawal risk and assessment**
- Opioid intoxication causes central nervous system (CNS) depression and death may result secondary to respiratory failure.
- Classic opioid toxidrome includes:
 - CNS depression
 - Respiratory depression
 - Miosis (pupil constriction)
- Some opioids may have unexpected toxicity
 - Methadone use is associated with QT prolongation.¹⁷
 - Pethidine¹⁸ / Fentanyl¹⁹ use may cause serotonin syndrome.

6.3 Overdose management

- Patient care includes:
 - airway, breathing, circulation management refer to [SCGH Medical Emergency Team](#): Code Blue
 - Naloxone is the specific antidote. Titrated doses of intravenous naloxone in line with [SCGOPHCG Naloxone Guideline](#)
 - consult Clinical Toxicology services
 - consider Naloxone prefilled take-home syringes (Prenoxad) and nasal delivery device (Nyxoid) on discharge.

6.4 Opioid withdrawal

- Opioid withdrawal is the physiological response that results from the abrupt cessation or rapid reduction in opioid dose in an opioid dependent person or when the person is administered an opioid antagonist or partial agonist.
- The aim of withdrawal treatment is to support the patient and provide symptomatic relief from discomfort.
- The onset and duration of withdrawal depends on the half-life of the drug being taken.
- The severity and time course of withdrawal is dependent on the opioid used.
- Refer to Quick Reference Guide [Appendix 5](#) for common symptoms and time frame of opioid withdrawal

6.5 Opioid withdrawal management

- A comprehensive assessment and history are crucial in helping formulate a treatment plan including withdrawal management.
- Patients will be regularly monitored using the COWS.
- Assess for physical health harms which may occur with opioid use e.g., history of intravenous drug use has higher risk of BBVs.
- A liver function test may be beneficial for patients who have been on long term codeine and paracetamol combinations.

6.6 Clonidine use for management of withdrawal

- Clonidine is accepted for the management of opioid withdrawal symptoms in Australia. It can be used to reduce withdrawal symptoms such as nausea, vomiting, agitation and sweating¹⁶.
- Clonidine has little effect on reducing cravings; however, it does not produce opioid-like tolerance or dependence.
- Caution should be used when administering clonidine with other depressant medications e.g., benzodiazepines.
- Patients taking clonidine require monitoring for adverse effects including:
 - hypotension
 - bradycardia
 - sedation
- Prior to administering first dose of clonidine:
 - record baseline blood pressure and heart rate
- Clonidine must not be administered if:
 - systolic blood pressure < 90mmHg or
 - diastolic blood pressure < 50mmHg or
 - heart rate < 50 beats per minute or
 - clinical evidence of impaired circulation
- Initial clonidine test dose:
 - give 75 – 100micrograms orally and monitor blood pressure at 30 minutes post administration and document on the appropriate Observation and Response Chart
 - lying and standing blood pressure is measured pre and post
 - further clonidine is not recommended if systolic blood pressure is <90mmHg, diastolic blood pressure < 50mmHg, heart rate < 50 beats per minute
 - if no adverse side effects e.g., hypotension, dizziness a further 75 – 100micrograms oral dose of clonidine may be administered and continue treatment as per the table of symptomatic treatments below.
 - Maximum dose of 800micrograms daily.

Table 5. Symptomatic treatment management for mild opioid withdrawal

Symptoms	Suggested treatments
Nausea and vomiting	• Ondansetron PO/sublingual/IV 4-8mg TDS PRN. • Metoclopramide PO/IM/IV 5-10mg TDS/PRN Maximum 5 days of treatment • Prochlorperazine PO 5mg, every 4-6 hours PRN
Muscle aches and Pain	• Paracetamol PO 500-1000mg every 4-6 hours PRN. Maximum 4000mg Paracetamol in 24 hours • Ibuprofen PO 400mg every 6-8 hours PRN
Agitation and/or anxiety	• Clonidine PO 75 -150 microgram QID • Diazepam PO 5mg QID PRN
Insomnia	• Temazepam PO 10-20mg PRN at night
Abdominal cramps	• Hyoscine butylbromide PO/IV 10-20mg every six hours PRN • Paracetamol may also provide some relief
Diarrhoea	• Loperamide PO 2mg PRN after loose bowel motion. Maximum 16mg in 24 hours

6.7 Buprenorphine/naloxone (Suboxone) withdrawal protocol

- All patients undergoing this opioid detoxification within the hospital should be referred to and/or reviewed by the AOD service to ensure appropriate assessment of suitability, to provide wrap around support and referral to long-term opioid substitution therapy on discharge.
- For patients with a heavy opioid dependence who are likely to go through moderate to severe withdrawal whilst in a public hospital; specialist medical practitioners specified under **Section 2.5.8** of the Schedule 8 Medicines Prescribing Code can prescribe detoxification therapy.
- The patient must be made aware that treatment with Suboxone® for opioid detoxification will result in them being included on the Drugs of Addiction Record as a drug dependent person.
- It is the responsibility of the treating specialist prescriber to discuss the implications of detoxification treatment with the patient and provide the patient with a copy of the Department of Health Patient Information.
- Prior to commencing therapy, the specialist medical practitioner must complete and submit the 'Notification of the use of Suboxone®' Form for the treatment of opioid withdrawal in a hospital setting. This form is available by contacting Clinical Advisory Service (CAS) and requesting a copy of the notification form.
- Send the completed 'Notification of the use of Suboxone®' Form for the treatment of opioid withdrawal in a hospital setting to:
 - CPPAdmin@mhc.wa.gov.au or fax: 9471 0444.
 - after hours or on weekends – to Next Step On-Call Doctor
- If accepted by the Department, the applicant will become an approved detoxification prescriber for inpatients at the public hospital specified in the form.
- A notification form must be submitted for each patient.
- The detoxification period must be short term and may not exceed 5 days for any single period using the approved buprenorphine (Suboxone®) detoxification protocol (below).
- Changes to the approved protocol will require prior written authorisation from the Health Department and DACAS must be contacted for approval.

Table 6: Approved Buprenorphine (Suboxone) Protocol

Day	Suboxone® Film Regimen	Recommended Total Daily dose
Day 1	4mg at onset of withdrawal (*COWS score ≥ 12) Potential additional 2 to 4mg evening dose if necessary	4mg (maximum 8mg)
Day 2	4mg mane Potential additional 2 to 4mg evening dose if necessary	4mg (maximum 8mg)
Day 3	4mg mane Potential additional 2mg evening dose if necessary	4mg (maximum 6mg)
Day 4	2mg mane Potential additional 2mg evening dose if necessary	2mg (maximum 4mg)
Day 5	2mg if necessary	0mg (maximum 2mg)
Patient's COWS Score (see COWS form) Date: _____ Time: _____		
*COWS score should be assessed as ≥ 12 prior to Suboxone® administration		

- To reduce the risk of diversion the patient must place the prescribed dose of Suboxone® film underneath the tongue, taking care not to overlap multiple films and the nurse must supervise the patient for one minute after the dose is administered.
- Patients must not be supplied with Suboxone® to self-administer later, or at a different facility, or outside the observation of an approved health practitioner.
- During detoxification, a patient may not be prescribed, supplied, or administered any other S8 medicine. All reasonable steps must be taken to prevent patient access to other S8 medicines during this period. The authorised specialist must inform other S8 prescribers were known when detoxification commences.

7. Amphetamine substances

Psychostimulants /Amphetamine type substances (ATS)	A group of drugs which can stimulate the sympathetic and the central nervous system by increasing the synaptic concentration of excitatory neurotransmitters dopamine, norepinephrine and serotonin¹⁶. ATS can increase the release of these neurotransmitters, inhibit their reuptake or do a combination of both, producing a range of stimulatory effects. Refers to a group of drugs which includes • Amphetamines • Methylenedioxymethamphetamine (MDMA) • Cocaine hydrochloride A range of psychostimulants are also available for the treatment of various conditions. These may be used illicitly including: Methylphenidate (Ritalin ® and Concerta ®) Phentermine (Duromine ® and Metermine ®) Pseudoephedrine (Dimetapp ®, Sudafed ®)²
Amphetamines	Includes the four types of amphetamines: • Amphetamine • Dexamfetamine • Lisdexamfetamine • Methamphetamine
Sympathomimetic toxidrome	A group of symptoms associated with poisoning or exposure to a substance or class of substances including delusions, paranoia, tachycardia, hypertension, hypotension, hyperthermia, sweating and seizures.

7.1 Use of amphetamine substances

In general stimulants increase alertness, the sense of wellbeing and the pleasurable reinforcement experienced with many activities.

7.2 Signs and symptoms of intoxication and withdrawal

Refer Amphetamine Quick Reference Guide [See Appendix 6](#)

7.3 Withdrawal

- **Refer to AoD service 6457 6323 for withdrawal risk and assessment**
- A thorough assessment will include:
 - type of stimulant and its potency
 - dose, frequency, intensity, duration of use
 - mode of administration
 - poly-substance use

- comorbid physical or mental health issues
- Withdrawal can commence in the ED with identification and risk assessment and continued over the length of the admission.
- Withdrawal is not life threatening and it is safe for a patient to withdraw in the community setting.
- Withdrawal management aims to manage symptoms and engage the patient in psychoeducation and harm reduction.
- Patients who present solely to reduce or cease their psychostimulant use should be referred to the AoD service for a SBIRT inside working hours. A/H refer to AoD service for telephone/outpatient follow-up.

7.4 Withdrawal management

- Management of acute presentations includes:
 - identifying and treating symptoms/complications of psychostimulant misuse
 - providing a safe patient environment
 - patient observation and behavioural management
 - medical and psychiatric assessment as required
 - engagement with AOD services
- Consider Sympathomimetic toxidrome management.
- Refer to Section 9 for general nursing care requirements

7.5 Suggested pharmacotherapy treatments for symptomatic relief Amphetamine Withdrawal

Symptom	Suggested Pharmacotherapy
Muscle aches and pains	Paracetamol 1g PRNPO
Nausea and vomiting	Ondansetron PO/sublingual/IV 4-8mg TDS PRN. Metoclopramide PO/IM/IV 5-10mg TDS/PRN Maximum 5 days of treatment Prochlorperazine PO 5mg, every 4-6 hours PRN
Insomnia	5-10mg Diazepam Nocte up to seven days
Agitation and/or anxiety, restlessness, irritability	Diazepam PO 5mg up to QID PRN (adjust dose to clinical need and risks) Acute short-term use only, up to seven days
Thought Disturbance Paranoid Ideation Strange Beliefs Misperceptions Hallucination	Refer to C-L Psychiatry Service CNC for consideration of Olanzapine (Adult: 0435572264; Older Adult 65+ 0404828837) between 0830-1630 Monday to Friday.

8. Withdrawal chart cessation

AoD clinicians assess patients within clinical areas and make the decision that the patient reviewed is not in alcohol (or other substance) withdrawal, or that the ongoing use of diazepam is likely to put the patient at risk of harm.

- The AoD clinicians should make the decision to cease the relevant withdrawal chart. In addition to ceasing the relevant withdrawal chart, the companion doses of diazepam associated with the withdrawal chart must also be ceased on the medication chart as per AoD Standing Operating Procedure (SOP) 6.
- In the absence of an AoD clinical a medical officer can cease withdrawal chart if the patient has not displayed withdrawal features for 24 hours and refer to AoD service for follow-up.

If staff feel a patient needs to be recommenced on a withdrawal chart, please contact the AoD service for urgent referral.

Compliance, monitoring and evaluation

Medical and Nursing Directors will be responsible for identifying and implementing compliance monitoring strategies appropriate to their area. These compliance monitoring strategies may include evaluation, clinical incident reporting, auditing and/or feedback processes.

Identified compliance measures will be reported to the Service Safety Quality Risk Committee.

Related Documents

- Mental Health Act 2014
- Medicines and Poisons Act 2014
- Medicine and Poisons Regulations 2016
- Health (Notification of Stimulant Induced Psychosis) Regulations 2010
- Working together - Mental Health and Alcohol and Other Drug Engagement Framework 2018-2025

WA Health:

- [Alcohol and other drug withdrawal management policy \(MP0062/17\)](#)
- [Alcohol and Other Drugs Partnerships and Pathways Version 1.0](#)
- [Alcohol and Other Drugs Early Intervention Practice and Pathways Version 1.0](#)
- [Alcohol and Other Drugs Withdrawal Management Practice and Pathways Version 2.0](#)
- [Walk With Me Project](#)
- Next Step Drug and Alcohol Services. [A Brief Guide to Management of Alcohol and other Drug Withdrawal](#)
- [Recognising and responding to acute deterioration MP 017/22](#)
- [WA Health Consent to Treatment Policy MP 0175/22](#)
- [Medicines Handling policy MP 0139/20](#)
- [The Schedule 8 Medicines prescribing code](#)

Other:

- [Turning Point. Alcohol and other drug Withdrawal Guidelines.](#)
- [NSW Health. NSW Drug and Alcohol Withdrawal Clinical Practice Guidelines](#)
- [Queensland Health. Queensland Alcohol and Drug Withdrawal Clinical Practice Guidelines.](#)

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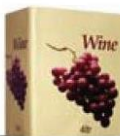
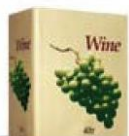
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Standards Applicable	NSQHS Standards (Version 2): 			
Aboriginal Health ISD Number:	2560			

Appendix 1: Standard drinks guide

NUMBER OF STANDARD DRINKS – BEER								
								
1.1 285ml Full Strength 4.8% Alc. Vol	0.8 285ml Mid Strength 3.5% Alc. Vol	0.6 285ml Low Strength 2.7% Alc. Vol	1.6 425ml Full Strength 4.8% Alc. Vol	1.2 425ml Mid Strength 3.5% Alc. Vol	0.9 425ml Low Strength 2.7% Alc. Vol	1.4 375ml Full Strength 4.8% Alc. Vol	1 375ml Mid Strength 3.5% Alc. Vol	0.8 375ml Low Strength 2.7% Alc. Vol
								
1.4 375ml Full Strength 4.8% Alc. Vol	1 375ml Mid Strength 3.5% Alc. Vol	0.8 375ml Low Strength 2.7% Alc. Vol	34 24 x 375ml Full Strength 4.8% Alc. Vol		24 24 x 375ml Mid Strength 3.5% Alc. Vol		19 24 x 375ml Low Strength 2.7% Alc. Vol	

NUMBER OF STANDARD DRINKS – WINE						
						
1.6 150ml Average Restaurant Serving of Red Wine 13.5% Alc. Vol	1 100ml Standard Serve of Red Wine 13.5% Alc. Vol	0.9 60ml Standard Serve of Port 18% Alc. Vol	1.4 150ml Average Restaurant Serving of White Wine 11.5% Alc. Vol	1 100ml Standard Serve of White Wine 11.5% Alc. Vol	1.4 150ml Average Restaurant Serve of Champagne 12% Alc. Vol	7.5 750ml Bottle of Champagne 12.5% Alc. Vol
						
8 750ml Bottle of Red Wine 13.5% Alc. Vol	43 4 Litres Cask Red Wine 13.5% Alc. Vol	21 2 Litres Cask Red Wine 13.5% Alc. Vol	7.5 750ml Bottle of White Wine 12.5% Alc. Vol	39 4 Litres Cask White Wine 12.5% Alc. Vol	19.5 2 Litres Cask White Wine 12.5% Alc. Vol	28 2 Litres Cask of Port 17.5% Alc. Vol

NUMBER OF STANDARD DRINKS – SPIRITS							
							
1 30ml High Strength Spirit Nip 40% Alc. Vol	22 700ml High Strength Bottle of Spirits 40% Alc. Vol	1.1 275ml Full Strength RTD* 5% Alc. Vol	1.2 330ml Full Strength RTD* 5% Alc. Vol	2.6 660ml Full Strength RTD* 5% Alc. Vol	1.5 275ml High Strength RTD* 7% Alc. Vol	1.8 330ml High Strength RTD* 7% Alc. Vol	3.6 660ml High Strength RTD* 7% Alc. Vol
							
1 250ml Full Strength Pre-mix Spirits 5% Alc. Vol	1.2 300ml Full Strength Pre-mix Spirits 5% Alc. Vol	1.5 375ml Full Strength Pre-mix Spirits 5% Alc. Vol	1.7 440ml Full Strength Pre-mix Spirits 5% Alc. Vol	1.4 – 1.9 250ml High Strength Pre-mix Spirits 7% – 10% Alc. Vol	1.6 300ml High Strength Pre-mix Spirits 7% Alc. Vol	2.1 375ml High Strength Pre-mix Spirits 7% Alc. Vol	2.4 440ml High Strength Pre-mix Spirits 7% Alc. Vol

Appendix 2: Alcohol Withdrawal Management

Signs of intoxication	Signs of withdrawal	Mild to moderate withdrawal symptoms	
Poor motor coordination Impaired gait Slurred speech Disinhibition Poor concentration Mood instability Altered level of consciousness	Onset 6-24 hours after last drink	Agitation, anxiety, disturbed sleep, nausea, restlessness, sweats, tachycardia, hypertension, tremor, raised temperature	
	Breath Alcohol Level may still be elevated at onset	Severe withdrawal symptoms	
		Worsening of above symptoms plus: delirium tremens, extreme agitation, confusion, paranoia, hallucinations, seizures (usually within the first 48 hours), death	
	Lasts for 3-5 days but can be as long as 10 days	Wernicke's encephalopathy	
		Symptoms may include confusion (70%), ataxia, nystagmus, hypothermia	
Assessment / management tools (once diagnosis established)			
Clinical assessment and examination Clinical tool examples: Clinical Institute Withdrawal Assessment of Alcohol Scale (CIWA-Ar)			
Acute withdrawal treatment models			
Supportive care	Benzodiazepines (Grade B): Select one of...		
	Fixed dose regimen (Grade D)	Symptom-triggered sedation (Grade D)	Diazepam loading (Grade D)
Symptomatic care: Adequate hydration Antiemetics Paracetamol Consider: Vitamin B complex Multivitamins Folic acid Magnesium In severe liver disease (Childs-Pugh C) use lorazepam in preference to diazepam – monitor for sedation and confusion. Lorazepam 1 mg PO is equivalent to diazepam 5 mg PO. Seek specialist AOD service advice in patients with decompensated liver cirrhosis where benzodiazepines may contribute to hepatic encephalopathy	Suitable for patients with mild to moderate withdrawal symptoms who have good support, no complicating factors and no history of seizures Diazepam is the drug of choice for withdrawal symptoms An example regimen: Day 1: Diazepam PO 10 mg QID Day 2-3: Diazepam PO 5-10 mg TDS Day 4: Diazepam PO 5 mg BD Day 5: Diazepam PO 5 mg nocte then cease	As measured by assessment tool: Moderate withdrawal (CIWA-Ar 9 -14): diazepam PO 5-10 mg every 2-4 hours PRN (maximum 100mg/24hrs) Severe withdrawal (CIWA-Ar ≥ 15) diazepam PO 10-20mg every 2-4 hours PRN. (maximum 100mg/24hrs) Consider regular diazepam PO 10mg QID if history of withdrawal seizures	Initiate diazepam PO 20 mg on development of withdrawal symptoms Repeat doses of diazepam PO 10-20 mg every 2 hours until light sedation occurs or maximum 100mg/24hrs reached Ongoing titration against symptoms Consider regular diazepam PO 10mg QID if history of withdrawal seizures

Wernicke's encephalopathy prevention and treatment	
Thiamine should be administered prior to any glucose or supplementary nutrition (Grade D) Check and correct electrolytes, including magnesium^d and phosphate to aid thiamine absorption (Grade D)	
Minimum dose for prophylaxis in all patients with acute alcohol withdrawal and normal mental state^e (Grade D)	Thiamine IV / IM 300 mg daily for 3 days. Continue 100 mg TDS orally until sober for 1 month Then 100 mg PO daily thereafter
Suspected Wernicke's encephalopathy OR acute alcohol withdrawal with altered mental state (Grade D)	Thiamine IV 250 – 500 mg TDS for 5 – 7 days. Continue 100 mg TDS orally until sober for 1 month Then 100 mg PO daily thereafter
Post withdrawal treatment options	
Anti-craving medications^f	Naltrexone^g (Grade A) Acamprosate (Grade A) Second-line agents
Support and relapse prevention	Support groups e.g. SMART recovery Ongoing counselling Alcoholics Anonymous Residential Rehabilitation Involve GP/AMS/CADS Involve appropriate support people

Appendix 3: Benzodiazepine withdrawal management

Signs of intoxication	Signs of withdrawal		
Poor motor coordination Slurred speech Disinhibition Low blood pressure Poor concentration Mood instability Sedation	Onset	Common	
	1-10 days after last dose Lasts 3-6 weeks (Depending on the half-life of the benzodiazepine)	Mental State: Anxiety / panic Insomnia / vivid dreams Restlessness / agitation Irritability Poor concentration / memory Depressed mood Feelings of depersonalisation	Physical Symptoms: Dry retching / Nausea Decreased appetite Aches and pains Headaches Palpitations Tremor Blurred vision Increased temperature Ataxia Menstrual changes
		Uncommon (severe)	
		Delirium, Delusions, Paranoia, Hallucinations, Catatonia, Seizures	
Assessment / management tools (once diagnosis established)			
Clinical assessment and examination			
Acute withdrawal treatment models			
General principles		Inpatient management	
Tolerance to benzodiazepines develops quickly Most benzodiazepine withdrawal is managed in the community with GP or specialist AOD service involvement Refer patients on an opioid treatment program back to their prescribers Do not prescribe to patients with whom you are not familiar Check the Medicare prescription shopping hotline Phone 1800 631 181 OR WA Health S8 prescriber information service Phone 08 9222 4424		Acute withdrawal is managed by dose stabilisation whilst in hospital and then referred for management in the community. Clinical judgement on dose is required as history may be unreliable. Specialist advice is recommended for polysubstance users If there is no hepatic impairment, convert daily dose to a diazepam-equivalent dose. Reduce this by 20-40% and prescribe this dose (up to 80 mg/day, whichever is lesser) in 4 divided doses daily. If history is uncertain, a conservative starting dose of 10mg QID PO is adequate, with dose adjustment based on symptoms Dose reduction should be slow aiming for 10% reduction every 1-2 weeks. Reduction rate can be slowed if preferred by the patient (Grade B)	
Post-withdrawal treatment options			
Ongoing counselling and support groups Involve GP/AMS and significant others			

Benzodiazepine Equivalents					
Generic Name	*Duration of Action	Approximate dose equivalent to 5mg Diazepam	Trade Names	Tablet Strengths	Schedule
Alprazolam	Short	0.5 – 1 mg	Alprax, Kalma, Xanax, Ralozam	0.25 mg, 0.5 mg 1 mg, 2 mg	Schedule 8 Controlled Drug
Bromazepam	Intermediate	3 – 6 mg	Lexotan	3 mg, 6 mg	S4R
Clobazam	Long	10 – 15 mg	Frisium	10 mg	S4R
Clonazepam	Long	0.25 – 0.5 mg	Rivotril, Paxam	0.5 mg, 2 mg	S4R
Diazepam	Long	5 mg	Antenex, Ducene, Ranzepam, Valium, Valpam	2 mg, 5 mg	S4R
Flunitrazepam	Long	1 – 2 mg	Hypnodorm	1 mg	Schedule 8 Controlled Drug
Lorazepam	Intermediate	0.5 – 1 mg	Ativan	1 mg, 2.5 mg	S4R
Nitrazepam	Long	5 mg	Alodorm, Mogadon	5 mg	S4R
Oxazepam	Short	15-30 mg	Alepam, Murelax, Serepax	15 mg, 30 mg	S4R
Temazepam	Short	10 – 20 mg	Normison, Temaze, Temtabs	10 mg	S4R
Non-benzodiazepine agents					
Zolpidem	Very Short	10 mg	Dormizol, Somidem, Stildem, Stilnox, Zolpibell	10mg (6.25mg & 12.5mg modified release)	S4R
Zopiclone	Very short	7.5 mg	Imovane, Imrest	7.5 mg	S4R
*Approximate duration of action			Approximate Time		
Very Short			Less than 6 hours		
Short			6 – 12 hours		
Intermediate			12 – 24 hours		
Long			Greater than 24hours		

Appendix 4: Cannabis withdrawal management

Signs of intoxication	Signs of withdrawal		
	Onset	Symptoms	Factors affecting severity
Euphoria Increased appetite Increased pulse Confusion Restlessness Hallucinations Delusions Anxiety / panic Paranoia	50-70% of dependent cannabis users will experience four or more withdrawal symptoms Commence on day 1 Peak at day 2-4 Symptoms usually last 2-3 weeks Occasional late development of anger and aggression up to two weeks after ceasing use	Anger / aggression Decreased appetite / weight loss Irritability Nervousness / anxiety Restlessness Sleep difficulties (including strange dreams) Cravings Sweating <i>Less common:</i> Depressed mood Paranoia	Psychiatric comorbidity Dose: amount, potency and preparation consumed History of aggression or violence Duration of current use and other past or current substance use history
Acute withdrawal treatment models			
General principles	Symptomatic management	Pharmacotherapy	Inpatient withdrawal
Most cannabis withdrawal can be managed in the community Symptomatic care is the mainstay for treatment Regular patient review is recommended Consider adding Nicotine Replacement Therapy for tobacco withdrawal (often spun with cannabis)	Simple analgesia Antiemetics Antispasmodics Hydration Calm environment Psychoeducation Decrease intake of caffeinated drinks	CONSIDER: Short course diazepam PO (5mg QID/PRN, max 5 days) to manage anxiety. Antipsychotic (eg Olanzapine 2.5-10 mg PO Daily) for paranoid or psychotic features (Grade C) Consult C-L Psychiatry	Generally, not required. Patients may undergo withdrawal whilst admitted as an inpatient for other reasons. Acute withdrawal while an inpatient should be managed in the same way as low medical withdrawal
Cannabis hyperemesis			
Recurrent nausea, vomiting and cramping abdominal pain due to cannabis use.		Capsaicin cream 0.025-0.075% applied topically to abdomen for symptoms of hyperemesis (Grade C)	
Post-withdrawal treatment options			
Consider an antidepressant for ongoing depressive symptoms (Grade D). Ongoing counselling GP / AMS and appropriate support people			

Appendix 5: Opioid withdrawal management

Signs of intoxication	Onset of withdrawal		
Euphoria (sense of wellbeing) Pinpoint pupils Sedation – ‘nodding off’ Decreased blood pressure Decreased pulse Slurred speech Decreased respiratory rate and oxygen saturations Sedation and Coma Death	<i>Short acting opioids:</i> Commences 6-24 hours after the last dose, peaks at 24-48 hours and resolves after 5-10 days	<i>Long acting (methadone or controlled release pharmaceutical) opioids:</i> Commences 24-48 hours after the last dose, peak severity less than for heroin withdrawal, but may be prolonged lasting 3-6 weeks	<i>Buprenorphine:</i> withdrawal is generally milder, commences within 3-5 days of the last dose and can last for several weeks
	Signs	Symptoms	
	Piloerection / Sweating Muscle twitching Vomiting / Diarrhoea Restlessness Yawning Rhinorrhoea Dilated pupils	Anorexia and nausea Abdominal pain Hot and cold flushes Bone, joint and muscle pain Insomnia and disturbed sleep Muscle cramps Intense craving for opioids	
Assessment/management tools (once diagnosis established)			
Clinical assessment and examination Clinical tools: Clinical Opioid Withdrawal Scale (COWS, see Appendix 1.5)			
Acute withdrawal treatment models			
General principles	Symptomatic management	Pharmacotherapy	
Most opioid withdrawal can be managed with GP or specialist AOD service involvement Seek specialist AOD advice and referral for opioid substitution programmes ^g eg methadone or Suboxone® It is recommended that pregnant women who are opioid dependent do not undergo opioid withdrawal due to risk of miscarriage or premature delivery	Simple analgesia Antiemetics Antispasmodics Antidiarrheal agents Hydration Calm environment Psycho-social education Consider: Clonidine for autonomic symptoms (Grade B) Example regimen: 75-150 microg clonidine QID PO and then tapering dose once peak symptoms pass. Omit or reduce dose if hypotensive or bradycardic	Sublingual buprenorphine / naloxone is effective for managing opioid withdrawal and may be considered for severe opioid withdrawal where clinical guidelines are in place (Grade B). Seek specialist AOD advice as an authorised prescriber is required. Where overdose prevention programmes exist, consider take home naloxone and peer naloxone training on discharge Continuation of Suboxone® on discharge requires prior approval from the Department of Health and must be requested by a CPOP prescriber – seek specialist AOD advice.	
Post withdrawal treatment options			
Risk of accidental overdose and death due to a reduction in tolerance to opioids needs to be clearly discussed should relapse occur Take home naloxone packs / information should be provided to support reduced risk Ongoing counselling and support groups Naltrexone ^h for relapse prevention GP/AMS/CADS/Significant others			

Appendix 6: Amphetamine Substances Withdrawal Management

Signs of intoxication	Withdrawal timeframe	Crash phase	Withdrawal phase	Extinction phase
Autonomic Increased blood pressure Increased body temperature Rapid or irregular heartbeat Excessive sweating Muscle rigidity Central Euphoria Agitation / confusion Thought disturbance Anxiety / panic Paranoia Psychotic features Acute behavioural disturbance	<i>Crash:</i> starts 12-24 hours after last dose and lasts 24-48 hours <i>Withdrawal:</i> starts 2-4 days after last use, peaks in severity over 7-10 days, then subsides over 2-4 weeks <i>Extinction:</i> lasts for weeks, and may persist for up to 12 months	Exhaustion Fatigue Generalised aches and pains <i>Sleep disturbance:</i> Increased sleep Insomnia Restlessness Flat mood or dysphoria Anxiety / agitation Low level craving Thought disturbance usually masked during crash	Fatigue Anhedonia Generalised aches and pains Headache <i>Sleep disturbance:</i> Vivid dreams Insomnia <i>Fluctuating mood and energy:</i> Irritability Restlessness Anxiety / agitation Poor concentration and attention Strong cravings <i>Thought disturbance:</i> Paranoid ideation strange beliefs Misperceptions Hallucinations	<i>Episodic fluctuations in mood and energy levels:</i> Irritability Restlessness Anxiety / agitation Fatigue Lacking energy Anhedonia. Episodic cravings Disturbed sleep Gradual resumption of normal mood
Assessment / management tools (once diagnosis established)				
Clinical assessment and examination				
Clinical tool examples: currently there are no validated withdrawal scales for hospital settings				
Acute withdrawal treatment models				
General principles	Symptom management	Pharmacotherapy	Inpatient withdrawal	
Most amphetamine withdrawal can be managed in the community with good support Regular review may be required to monitor progress	Hydration Multivitamins Anti-Emetics Simple analgesia Calm environment Psycho-social education	CONSIDER: Low dose diazepam PO (5mg QID/PRN max 7 days) for agitation and poor sleep Antipsychotic (eg Olanzapine 2.5-10 mg PO daily) for paranoid or psychotic features (Grade C) Consult C-L Psychiatry	Acute withdrawal whilst admitted should be managed supportively	
Post-withdrawal treatment options				
Ongoing counselling and support/groups, GP / AMS Residential rehabilitation / therapeutic communities Consider an antidepressant if ongoing depressive symptoms (Grade D).				

Appendix 7: Evidence of classification schemes

Identification and assessment of evidence is best achieved through systematic reviews where all available evidence is assessed for its applicability to the clinical question being considered, reviewing the evidence for bias, and summarising the findings.

The type of evidence required is dependent on the question under consideration. Where efficacy of treatment interventions is the issue, as was the case in this literature review; randomised, controlled trials are most relevant.

The summarised evidence is then categorised based on its susceptibility to bias, which is often related to study design, or analysis of the findings e.g. selection bias (sample is not representative of the population) and confirmation bias (interpreting data to prove a predetermined assumption).

NHMRC Levels of evidence

Grade	Evidence Description	Recommendation
A	One or more level I studies with a low risk of bias or several level II studies with a low risk of bias. All studies consistent. Very large clinical impact. Population/s studied in body of evidence are the same as the target population for the guideline. Directly applicable to Australian healthcare context.	Excellent Body of evidence can be trusted to guide practice. Recommendation based on high quality evidence. Strongly recommended for implementation.
B	One or two level-II studies with a low risk of bias or a SR/several level III studies with a low risk of bias. Most studies consistent and inconsistency may be explained. Substantial clinical impact. Population/s studied in the body of evidence are like the target population for the guideline. Applicable to Australian healthcare context with few caveats.	Good Body of evidence can be trusted to guide practice in most situations. Recommendation based on good evidence. Strongly recommended for implementation.
C	One or two level III studies with a low risk of bias, or level I or II studies with a moderate risk of bias. Some inconsistency reflecting genuine uncertainty around clinical question. Moderate clinical impact. Population/s studied in body of evidence differ to target population for guideline, but it is clinically sensible to apply this evidence to target population. Probably applicable to Australian healthcare context with some caveats.	Satisfactory Body of evidence provides some support for recommendation(s) but care should be taken in its application. Recommendation based on supportive evidence and a strong theoretical rationale. Recommended for implementation.
D	Evidence level 3 or 4, or Extrapolated evidence from studies rated as 2+, or Formal consensus.	Poor Body of evidence is weak, and recommendation must be applied with caution. Recommendation based on limited, inconsistent, or extrapolated evidence. Recommendation supported by expert opinion. Recommended for implementation.
D (GPP)	A good practice point (GPP) is a recommendation for best practice based on the experience of the Guideline Development Group.	GPP Evidence limited or non-existent. Recommendation based on current expert opinion and trends in clinical practice. Recommended for implementation.

Appendix 8: Drugs with the potential to interact with alcohol

Medication	Type of Interaction
Sedative–hypnotics: Benzodiazepines, barbiturates	Acute alcohol consumption: • potentiates the central nervous system depressant effects of benzodiazepines and barbiturates. • decreases availability of barbiturates through hepatic enzyme induction, decreasing their effect. • Increases risk of cognitive impairment, respiratory depression, and overdose.
Anaesthetic agents	Chronic alcohol consumption: • Increases the dose of propofol required to induce anaesthesia • Increases the risk of liver damage by anaesthetic gases enflurane and halothane.
Opioid analgesics	• Alcohol increases sedative effect. • Increased risk of cognitive impairment, respiratory depression, and opioid overdose.
Tricyclic antidepressants	• Acute alcohol consumption increases risk of sedation and orthostatic hypotension (sudden drop in blood pressure upon standing up).
Antihistamines	• Alcohol potentiates the central nervous system depressant effect of sedating antihistamines, especially in elderly people.
Antipsychotic medication: • Phenothiazines • Olanzapine	• Acute alcohol consumption increases sedative effects, impairs coordination, and may result in liver impairment. • Alcohol increases sedation and risk of hypotension.
Oral hypoglycaemic agents: • Sulfonylurea compounds	• Diabetics on sulfonylureas should be advised not to drink. • Acute alcohol ingestion prolongs availability of hypoglycaemic agents leading to hypoglycaemia. • Hypoglycaemia may also occur if there is malnutrition or depletion of glycogen stores. • Chronic alcohol administration decreases the availability of hypoglycaemic agents with risk of hyperglycaemia.
Anticonvulsants: • Phenytoin	• Acute alcohol consumption increases availability of Phenytoin increasing risk of side effects. • Chronic alcohol consumption decreases anticonvulsant effect of Phenytoin.
Histamine H2 receptor antagonists: • Cimetidine	• Inhibit gastric alcohol dehydrogenase and increase the rate of gastric emptying. This may increase blood alcohol concentration.
Oral anticoagulants: • Warfarin	• Acute alcohol consumption increases Warfarin's availability, increasing risk of haemorrhage. • Chronic alcohol consumption reduces availability of Warfarin, decreasing its anticoagulant effect.
Non-narcotic analgesics: • Aspirin, NSAIDs • Paracetamol	• Alcohol increases the risk of gastrointestinal bleeding. • Chronic alcohol consumption increases risk of liver damage with paracetamol overdose.
Antibiotics: • Metronidazole	• Metronidazole appears to inhibit alcohol dehydrogenase and other alcohol oxidising enzymes. • Patients should be advised not to take alcohol during therapy or for at least 48 hours afterwards because of the possibility of a disulfiram-like (antabuse effect).

